SCML — SITE COMPLICATION MONITORING LOG

Study / Protocol	Site / Code	SCML Record ID / Revision	SCML owner / role
Northbridge Shoulder Feasibility Study / NB-Shoulder-01	Northbridge Orthopaedic Site / NB-014	SCML-NB014-2026-10-01 / r1	Laura Gomez / Site QA Coordinator
Device / System	**Procedure Family / Anatomical Field**	**Record Opened**	**Last Updated**
Northbridge Reverse Shoulder System / NBR-RSA	Reverse shoulder arthroplasty / shoulder	06 Jul 2026	30 Oct 2026
Applicable CIP / Safety Procedure / Current Version		**Study Category Set / Version (if used)**	**CSST Review Period / Next Review Date**
NB-Shoulder-01 CIP v2.0 / NB-SAF-02 v1.1		NB-SCAT-01 v1.0	Oct 2026 / 05 Nov 2026

A Purpose and use

This log brings together the site's complications, adverse events (AEs), adverse device effects and device deficiencies (DDs) for review across cases. **The Safety and Adverse Effect Reporting Template (SART) provides the individual safety case.** The Intraoperative Checklist and Device Log (ICDL) supplies the procedure and device context.

Output: an indexed set of events with a consistent surveillance profile, current source references and linked follow-up. The Complication Signal Surveillance Template (CSST) uses this index to assess patterns; the Principal Investigator (PI) reads its status through the PI Oversight Template (POT).

The SART retains the clinical chronology, causality assessment and reporting details. Record the current classifications and source links here, with enough context to compare cases. Urgent care and safety notification follow the applicable procedure immediately, without waiting for this log or an aggregate review.

B Recording and Source-Link Guide

Step	What to record here	Source and boundary
Link the event	C: assign a unique SCML Event ID and enter the SART case ID and specific event reference first. Use participant and surgeon codes.	Keep the same Event ID in C and D. Reference the ICDL and imaging source where relevant; do not copy clinical narratives.
Apply a consistent profile	D: select the event type, one primary surveillance category and timing group. Copy the current Clavien-Dindo grade, relationship context and DD finding from the SART.	Use Pending where information is unresolved. Use the same category labels across entries; any Other code comes from the study category set.
Review and follow through	E: cite events or a group and the receiving record. F: check the status and conclusion of an existing action or handoff.	CSST holds rates and the formal signal decision. The SART or linked quality record retains case follow-up, action evidence and effectiveness assessment.

Open the index while the source case is still being assessed. Update the existing entry when new information changes the same event. A distinct event receives a new Event ID. AE and DD pathways may both apply to one entry; this alone does not create a second clinical event. The Configuration change category refers to an unplanned device configuration change.

Grade	Clavien-Dindo definition	Example in shoulder arthroplasty
I	Any deviation from the normal postoperative course without drugs, surgery or radiological intervention beyond antiemetics, antipyretics, analgesics, diuretics, electrolytes and physiotherapy; wounds opened at the bedside.	Superficial wound problem treated with dressings; transient nerve palsy resolving without treatment.
II	Needs drug treatment beyond grade I, blood transfusion or parenteral nutrition.	Wound infection treated with antibiotics; thromboembolism treated with anticoagulation.
IIIa	Needs a surgical, endoscopic or radiological intervention not under general anesthesia.	Closed reduction of a dislocation under sedation; aspiration of a hematoma.
IIIb	Needs an intervention under general anesthesia.	Open reduction or revision for instability; liner exchange; washout for deep infection.
IVa	Life-threatening, single organ dysfunction needing intermediate or intensive care.	Pulmonary embolism needing intensive care.
IVb	Life-threatening, multiorgan dysfunction.	Septic shock from a periprosthetic infection.
V	Death of the participant.	

The grade follows the treatment the event needed and is set in the SART. Add the suffix d when a disability remains at discharge (for example IIIb-d). A device deficiency without an adverse event takes N/A.

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Event and source index

Study / Site Code	SCML Record ID / Revision	Sheet updated (date)	Sheet No. / Total (C)
NB-Shoulder-01 / NB-014	SCML-NB014-2026-10-01 / r1	30 Oct 2026	1 / 1

C Event and Source Index

Enter one row per distinct event or deficiency. **SART case ID / event reference** links the row to its individual source. Use the same clinical term or study term/code as the source. Record the event date and days from the procedure; enter N/A when the interval does not apply. New information about an existing event updates that entry and its source link.

SCML Event ID	SART Case ID / Event Reference	Participant Code	Event Term / Study Code	Event Date / Days from Procedure	Device / Component Code	Surgeon Code	ICDL / Procedure Reference	Imaging Reference
SCML-001	SART-NB014-004 / AE-01	NB014-0004	Dislocation	22 Sep 2026 / day 20	GS-36 / BP-A	S01	ICDL-NB014-004	IMG-NB014-004-F01
SCML-002	SART-NB014-006 / DD-01	NB014-0006	Trial locking mechanism jam	25 Sep 2026 / day 0	Trial inserter TI-02	S02	ICDL-NB014-006	Not indicated
SCML-003	SART-NB014-009 / AE-01	NB014-0009	Dislocation	16 Oct 2026 / day 15	GS-36 / BP-A	S01	ICDL-NB014-009	IMG-NB014-009-F01
SCML-004	SART-NB014-011 / AE-01	NB014-0011	Wound infection	20 Oct 2026 / day 15	None implicated	S02	ICDL-NB014-011	Not indicated
SCML-005	SART-NB014-014 / AE-01 + DD-01	NB014-0014	Intraoperative calcar fracture	25 Oct 2026 / day 0	Broach BR-12	S02	ICDL-NB014-014	IMG-NB014-014-OR01

Use the same Event ID in D and mark negative or pending findings explicitly. Keep source details in SART/ICDL; date and initial corrections.

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Structured surveillance profile

Study / Site Code	SCML Record ID / Revision	Sheet updated (date)	Sheet No. / Total (D)
NB-Shoulder-01 / NB-014	SCML-NB014-2026-10-01 / r1	30 Oct 2026	1 / 1

D Structured Surveillance Profile

Use the Event ID from C. Select one event type, primary category and timing group. **Copy the Clavien-Dindo grade, relationship contexts, the DD finding and assessment status from the current SART**; multiple relationship contexts may apply. Record the date/initials checked and use Pending while unresolved.

SCML Event ID	Event Type (from SART)	Primary Surveillance Category	Timing Group	Grade (Clavien-Dindo) (from SART)	Relationship Context (from SART)	Observed DD? (from SART)	SART Assessment Status / Source Check
SCML-001	☒ AE ☐ DD without AE ☐ AE + DD ☐ Pending	☒ Instability / dislocation ☐ Fracture ☐ Infection / wound ☐ Neurologic / vascular ☐ Device performance ☐ Configuration change ☐ Other code: _____ ☐ Pending	☐ Intraoperative ☒ Postoperative ☐ Other / N/A ☐ Pending	☐ I ☐ II ☒ IIIa ☐ IIIb ☐ IVa ☐ IVb ☐ V ☐ N/A ☐ Pending	☒ Device ☒ Procedure ☐ Operator ☐ Patient ☐ Undetermined	☐ Yes ☒ No ☐ Pending	☐ Pending ☐ Preliminary ☒ Final Date / initials: 30 Oct / LG
SCML-002	☐ AE ☒ DD without AE ☐ AE + DD ☐ Pending	☐ Instability / dislocation ☐ Fracture ☐ Infection / wound ☐ Neurologic / vascular ☒ Device performance ☐ Configuration change ☐ Other code: _____ ☐ Pending	☒ Intraoperative ☐ Postoperative ☐ Other / N/A ☐ Pending	☐ I ☐ II ☐ IIIa ☐ IIIb ☐ IVa ☐ IVb ☐ V ☒ N/A ☐ Pending	☒ Device ☐ Procedure ☐ Operator ☐ Patient ☐ Undetermined	☒ Yes ☐ No ☐ Pending	☐ Pending ☐ Preliminary ☒ Final Date / initials: 30 Oct / LG
SCML-003	☒ AE ☐ DD without AE ☐ AE + DD ☐ Pending	☒ Instability / dislocation ☐ Fracture ☐ Infection / wound ☐ Neurologic / vascular ☐ Device performance ☐ Configuration change ☐ Other code: _____ ☐ Pending	☐ Intraoperative ☒ Postoperative ☐ Other / N/A ☐ Pending	☐ I ☐ II ☐ IIIa ☒ IIIb ☐ IVa ☐ IVb ☐ V ☐ N/A ☐ Pending	☒ Device ☒ Procedure ☐ Operator ☐ Patient ☐ Undetermined	☐ Yes ☒ No ☐ Pending	☐ Pending ☒ Preliminary ☐ Final Date / initials: 30 Oct / LG
SCML-004	☒ AE ☐ DD without AE ☐ AE + DD ☐ Pending	☐ Instability / dislocation ☐ Fracture ☒ Infection / wound ☐ Neurologic / vascular ☐ Device performance ☐ Configuration change ☐ Other code: _____ ☐ Pending	☐ Intraoperative ☒ Postoperative ☐ Other / N/A ☐ Pending	☐ I ☒ II ☐ IIIa ☐ IIIb ☐ IVa ☐ IVb ☐ V ☐ N/A ☐ Pending	☐ Device ☒ Procedure ☐ Operator ☒ Patient ☐ Undetermined	☐ Yes ☒ No ☐ Pending	☐ Pending ☒ Preliminary ☐ Final Date / initials: 30 Oct / LG
SCML-005	☐ AE ☐ DD without AE ☒ AE + DD ☐ Pending	☐ Instability / dislocation ☒ Fracture ☐ Infection / wound ☐ Neurologic / vascular ☐ Device performance ☐ Configuration change ☐ Other code: _____ ☐ Pending	☒ Intraoperative ☐ Postoperative ☐ Other / N/A ☐ Pending	☐ I ☐ II ☐ IIIa ☒ IIIb ☐ IVa ☐ IVb ☐ V ☐ N/A ☐ Pending	☒ Device ☒ Procedure ☐ Operator ☐ Patient ☐ Undetermined	☒ Yes ☐ No ☐ Pending	☐ Pending ☒ Preliminary ☐ Final Date / initials: 30 Oct / LG

Relationship contexts reflect the current source assessment. Categories and DD flags are not additional events; follow the CSST counting rule.

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Local review and linked follow-up

Study / Site Code	SCML Record ID / Revision	Sheet updated (date)	Sheet No. / Total (E/F)
NB-Shoulder-01 / NB-014	SCML-NB014-2026-10-01 / r1	30 Oct 2026	1 / 1

E Local Review and Handoff

Log one finding per event or group with a unique Review ID. Select the receiving record and record the decision, linked ID, owner and due date. Use the same Review ID in F. Signal decisions remain in the CSST.

Review ID / Date	Event IDs / Group Reference	Review Finding / Reason for Handoff	Receiving Record(s)	Decision / Linked Record ID / Owner / Due Date
R001 25 Sep 2026	SCML-002	Observed device deficiency; no AE recorded.	☒ Safety / SART ☐ CSST ☐ SLCT ☐ SRR ☐ CAPA ☐ POT	SART-NB014-006 / deficiency route. LG / due 30 Sep 2026.
R002 16 Oct 2026	SCML-001 + SCML-003	Two dislocations; same component codes and surgeon S01.	☐ Safety / SART ☒ CSST ☒ SLCT ☐ SRR ☐ CAPA ☒ POT	CSST-NB014-2026-10-02 / SLCT-NB014-S01-R02 / POT-NB014-2026-10-03. LG / due 05 Nov 2026.
R003 20 Oct 2026	SCML-004	Wound event; case assessment and follow-up remain open.	☒ Safety / SART ☐ CSST ☐ SLCT ☐ SRR ☐ CAPA ☐ POT	SART-NB014-011 follow-up. SC / due 03 Nov 2026.
R004 25 Oct 2026	SCML-005	Intraoperative fracture with a recorded device deficiency.	☒ Safety / SART ☐ CSST ☐ SLCT ☐ SRR ☐ CAPA ☐ POT	SART-NB014-014 / AE + DD routes. SC / due 02 Nov 2026.

F Linked Action and Handoff Follow-up

Check the existing action in its owning record and cite its outcome and effectiveness conclusion, where applicable. Route a new, overdue or ineffective action through E. Keep a handoff open until the required follow-up is documented.

Review / Event IDs	Existing Action / Handoff Record ID	Current Status	Owner / Due Date	Source Conclusion / Evidence Reference / Checked Date and Initials
R001 / SCML-002	SART-NB014-006	☐ Open / on time ☐ Overdue ☒ Closed ☐ Not assessable	LG / 30 Sep 2026	Return acknowledged; source case closed. SART-NB014-006 G / 30 Sep 2026 / LG.
R002 / SCML-001 + 003	CSST-NB014-2026-10-02 / SLCT-NB014-S01-R02	☒ Open / on time ☐ Overdue ☐ Closed ☐ Not assessable	LG / 05 Nov 2026	Triggered review open; no signal conclusion yet. CSST-NB014-2026-10-02 D / 30 Oct 2026 / LG.
R003 / SCML-004	SART-NB014-011 / follow-up	☒ Open / on time ☐ Overdue ☐ Closed ☐ Not assessable	SC / 03 Nov 2026	Assessment pending. SART-NB014-011 F / checked 30 Oct 2026 / LG.
R004 / SCML-005	SART-NB014-014 / AE + DD	☒ Open / on time ☐ Overdue ☐ Closed ☐ Not assessable	SC / 02 Nov 2026	AE/DD follow-up pending. SART-NB014-014 F / checked 30 Oct 2026 / LG.

G Current Index Status and Review Conclusion

Select the highest-priority status: Escalated review active → Control action active → Follow-up or assessment pending → Index current. Cite affected Event IDs and records below. Routine CSST review is not escalation; case closure remains in the SART.

Current Index Status	Select One / Basis	Status Date / Next Review
Index current / no pending item	☐ Entries are reconciled with their sources; no pending assessment, action or handoff is known.	
Follow-up or assessment pending	☐ A source assessment, case follow-up or reconciliation is pending; no higher-priority status applies.	
Control action active	☐ A linked corrective or procedural control action remains open; no escalated review status applies.	
Escalated review active	☒ An urgent or overdue safety handoff, or a triggered signal review, remains open.	30 Oct 2026 Next: 05 Nov 2026

Affected Event IDs / Linked Records / Status Basis:

R002: SCML-001 + SCML-003 linked to CSST-NB014-2026-10-02 / SLCT-NB014-S01-R02; review open, due 05 Nov. R003/R004 remain in SART-NB014-011 and SART-NB014-014. No confirmed signal recorded.

H Accountable Review and Controlled Record Location

The reviewers confirm that the index was checked against the linked source records and that outstanding handoffs remain identifiable. This review does not replace the investigator's case assessment or any sponsor safety decision. One person may hold more than one site function.

SCML owner / primary reviewer

Name / role: Laura Gomez / coordinator
Signature: Laura Gomez
Date: 30 Oct 2026

Clinical / surgical reviewer (if required)

Name / role: Dr Sofia Chen / surgeon
Signature: Sofia Chen
Date: 30 Oct 2026

Principal Investigator / accountable approver

Name: Dr Sofia Chen
Signature: Sofia Chen
Date: 30 Oct 2026

Next review and controlled record location

Review date: 05 Nov 2026
Record location: Site file / Safety / SCML-NB014-2026-10-01
Current revision: r1